

The Green Mountain Care Board: Vermont's Regulatory Architecture

Vermont's healthcare transformation would be impossible without an institutional actor that most other states do not have. The Green Mountain Care Board — a five-member independent state body created in 2011 — is the regulatory engine that converts Vermont's legislative mandates into binding operational obligations for its hospital system. Understanding the GMCB is prerequisite to understanding why Vermont can attempt what other states cannot.

Vermont's hospital budgets have been subject to state review since 1983, but GMCB's assumption of regulatory authority beginning in fiscal year 2013 qualitatively changed the nature of that oversight. The GMCB does not merely review hospital budgets — it establishes them. Under Vermont statute, each hospital must operate within the budget the Board sets. When hospitals exceed those budgets, the GMCB can enforce by reducing future commercial rate approvals. When hospitals face financial distress, the GMCB Chair meets directly with hospital leadership. When the Legislature mandates transformation planning under Acts 167 and 68, GMCB is the institutional receptor for the resulting analysis and the body responsible for translating policy architecture into hospital-level operational requirements.

The practical significance of this regulatory architecture extends beyond Vermont's 14 hospitals. GMCB's annual budget review process, conducted entirely in public meetings under Vermont's Open Meeting Law with mandatory participation from the Office of the Health Care Advocate, has produced a documentary record of Vermont's hospital system that is unmatched in American health policy. The GMCB's Adaptive Insights database contains actual and budgeted financial performance for all 14 hospitals from 2013 forward. Its monthly monitoring reports track the fiscal trajectory of every institution in real time. Its published enforcement decisions — including the September 2024 enforcement of UVMMC's \$80.3 million FY23 budget overage — translate the abstract principle of regulatory authority into concrete financial consequences.

For FY25, GMCB approved \$3.7 billion in system-wide hospital spending — a 4.1% increase after hospitals collectively requested 8%. The Board also enforced FY23 budget overages against UVMMC and RRMC, reducing future commercial rate approvals as a consequence. Both decisions illustrate the dual function GMCB serves in Vermont's transformation: it is simultaneously a cost-control instrument — protecting Vermonters from commercial rate growth that exceeds what the financial evidence justifies — and a transformation lever, using its regulatory authority to drive hospitals toward population health models that require them to reduce expensive utilization rather than maximize it.

The table below summarizes GMCB's principal regulatory functions and their relationship to Vermont's transformation agenda.

GMCB function	What it does and why it matters for transformation
Hospital budget review	Annually establishes Net Patient Revenue (NPR) growth limits for all 14 hospitals. For FY25, GMCB approved \$3.7 billion in hospital spending — a 4.1% system-wide increase after hospitals requested 8%. This regulatory compression is both a

	cost-control tool and a structural forcing function that drives hospital attention to efficiency and alternative revenue models.
Budget enforcement	When hospitals exceed approved budgets without prior adjustment requests, GMCB can enforce by reducing future commercial rate approvals. In FY23, UVMMC exceeded its budget by \$80.3M and RRMC by \$11.1M — GMCB enforced both, reducing future commercial rates. This enforcement authority is what distinguishes Vermont's regulatory model from purely advisory state oversight.
Monthly performance monitoring	All 14 hospitals submit monthly financial performance reports to GMCB throughout the year. This continuous monitoring capability is the operational substrate for Vermont's transformation accountability — it is what makes the Oliver Wyman findings actionable rather than merely descriptive.
Reference-based pricing (RBP)	Act 68 mandates reference-based pricing beginning FY2027, with GMCB setting the methodology. RBP will cap commercial hospital rates as a percentage of Medicare, eliminating Vermont hospitals' current ability to charge commercial payers three to four times Medicare rates. GMCB's existing rate review infrastructure is what makes mandatory RBP implementable.
Global budget design	GMCB will set prospective global hospital budgets beginning FY2028-2030, in coordination with the AHEAD Model. The active analytics vendor procurement underway in 2025-2026 is GMCB building the technical infrastructure for global budget methodology — this is the highest-risk gap in Vermont's current transformation architecture.
Hospital sustainability planning	Under Act 167, GMCB commissioned the Oliver Wyman analysis and is the institutional receptor for its recommendations. GMCB's public meeting record — including the September 18, 2024 Oliver Wyman presentation — is the documentary foundation for Vermont's transformation case and has no equivalent in most other states.

Figure 2.1. GMCB regulatory functions and transformation significance. Source: GMCB Hospital Budget Review Guide; Acts 167 and 68; GMCB Annual Report 2024.

GMCB's institutional design explains a specific feature of Vermont's transformation that is analytically important: mandatory authority precedes payment reform deployment. The voluntary ACO era produced modest results because the highest-cost providers could decline to participate. GMCB's mandatory budget authority — its ability to set rates rather than merely recommend them — is the structural condition that makes Vermont's mandatory reference-based pricing and global budgeting achievable. Other states attempting payment reform without a comparable regulatory body are, in the framework's terms, attempting Economics pillar transformation without the Policy pillar architecture that gives payment reform enforcement teeth.

The Fourteen Hospitals: A System Portrait

Vermont's 14 community hospitals are not a homogeneous set of institutions facing identical challenges. They are a differentiated system — ranging from the state's sole academic medical center at UVMMC, serving as Vermont's Level One Trauma Center, children's hospital, and only site for kidney transplantation, to Critical Access Hospitals serving rural communities of fewer than 5,000 people. Understanding the typology of this system — what each hospital is, where it is in its financial and operational trajectory, and what its transformation plan reveals about its priorities and constraints — is essential context for every subsequent chapter's argument.

Of Vermont's 14 hospitals, the University of Vermont Health Network comprises UVMHC, CVMC, and Porter Hospital as system hospitals. Dartmouth Health affiliates include SVMC and Mt. Ascutney. The remaining hospitals — Gifford, Copley, NCH, NVRH, BMH, Springfield, RRMC, Grace Cottage, and NMC — operate as independent community institutions or loose collaborative members, and it is these independent hospitals that face the most acute financial fragility. Nine of Vermont's 14 hospitals reported operating losses in FY2023, with the worst reaching negative 8.9%. Oliver Wyman's trajectory analysis projects that 13 of 14 would be in losses by 2028 absent structural intervention.

The January 2026 transformation plans submitted to AHS under Act 68 provide a uniquely granular view of where each hospital stands. These are not aspirational strategic plans — they are operational documents produced in response to a statutory mandate, reviewed by AHS staff, and directly tied to Rural Health Transformation Program grant funding. The AHS engagement tracking data for each hospital — plan quality ratings, grant status, identified concerns, and next steps — provides a real-time picture of transformation readiness that no external analysis can replicate.

The following table characterizes all 14 hospitals by type, transformation plan status, primary focus, and key challenges as documented in the January 2026 plan submissions and AHS tracking data.

Hospital	Type	Grant	Primary transformation focus	Key challenges / concerns
UVM Medical Center (UVMHC)	Academic hub	Not eligible	Technology solutions for acuity management and transfers; statewide reference hub	Administrative cost \$3,826/adj. discharge vs. \$1,427 benchmark; budget overage enforcement (FY23)
Central Vermont Medical Center (CVMC)	Regional PPS	Awarded	Technology solutions for acuity management and patient transfers	Part of UVMHCN system; subject to system-level budget constraints
Porter Hospital	Regional PPS	Awarded	Technology solutions for acuity management and patient transfers (shared strategy with CVMC)	Part of UVMHCN system; FY23 budget overage of \$11M
Rutland Regional Medical Center (RRMC)	Regional PPS	Awarded	Hospice utilization improvement; dermatology in community; regional collaboration with SVMC	FY23 budget overage of \$11M (partially enforced); exploring regional service lines with SVMC
Southwestern Vermont Medical Center (SVMC)	Regional PPS	Awarded	RN Bridge to PCP program; \$29M cancer center; DaVinci robot; Epic EHR migration (\$30M); adolescent mental health unit	Locum provider costs; care coordination gaps; transport barriers for non-acute patients
Brattleboro Memorial Hospital (BMH)	Rural PPS	Awarded	Mobile Integrated Health (MIH) program formalization; FQHC Look-Alike feasibility; birthing and oncology sustainability	GMCB FY26 deep concern about solvency; operating losses FY22/FY24/projected FY25; executive bonus scrutiny

Springfield Hospital	Rural PPS	Needs follow-up	Clinical services redesign feasibility study; relationship with Northstar Health	Oncology and cardiology sustainability concerns; plan still not approved as of Jan 2026
Gifford Medical Center	CAH + FQHC	Awarded	Hospital-to-hospital transfer program with UVMHC/DHMC; NECHN GPO (\$793K projected savings); shared radiology	Targeting avg. daily census of 20 for sustainability; tele-radiology vendor capacity constraints
Copley Hospital	CAH	Awarded	Consulting services (KH); primary care access expansion	Plan quality concerns; GMCB required modifications to FY25 budget; sustainability of changes to be discussed
North Country Hospital (NCH)	CAH	Awarded	Regionalization planning with NVRH; shared project manager and legal support; pulmonology; tele-neonatology	Limited IT staff; dependent on Vermont CIN shared services model for analytics capability
Northeastern Vermont Regional Hospital (NVRH)	CAH	Awarded	Regionalization planning with NCH; cardiology; tele-ICU	FY23 budget overage (not enforced); GMCB monitoring; limited workforce pipeline in Northeast Kingdom
Mt. Ascutney Hospital and Health Center (MAHHC)	CAH + FQHC	Awarded	Age-Friendly Program / geriatric COE; Collaborative Care Model (CoCM) via DHMC; shared CEO/CFO/COO with Valley Regional (\$600K savings)	EMR transition (Oct 2025) disrupted data collection; subsidizing pharmacy (\$50K/yr) after Woodstock pharmacy closure; residential care facility support (\$950K in FY25)
Grace Cottage Hospital	CAH	Awarded	\$24M primary care clinic capital campaign; same-day access clinic; swing bed program; inpatient bed evaluation	Denial reduction from \$76K to \$1,274 in one year (FY25 success); FY24 loss of \$4M reduced to ~\$1M in FY25; no DSH payments
Northwestern Medical Center (NMC)	CAH	Awarded	Innovative birthing services retention; shared cardiologist with Copley; oncology	Questions on value of marketing campaigns; schedule not yet confirmed as of Jan 2026

Figure 2.2. Vermont's 14 hospitals: typology, transformation status, and challenges. Source: AHS Hospital Tracking data (January 2026); individual hospital transformation plans (January 15, 2026); GMCB FY26 Budget Orders.

Several patterns in this data are analytically significant for the book's argument.

The administrative cost problem is concentrated at the top of the system. Oliver Wyman's finding that UVMHC's administrative cost per adjusted discharge was \$3,826 against a national benchmark

of \$1,427 — documented by AHS's November 2025 transformation report — is not merely an efficiency observation. It is evidence that Vermont's largest and most financially powerful hospital has structural cost problems that voluntary reform cannot address. GMCB's full enforcement of UVMHC's FY23 budget overage, and its decision to not approve NPR growth for BMH in FY26 while expressing deep concern about BMH's solvency, illustrate the regulatory tools available when transformation accountability is backed by statutory authority.

The regionalization logic is visible in the transformation plans but is not yet architecturally formed. NCH and NVRH are explicitly partnering to share a project manager, legal counsel, and subject matter expertise for regionalization planning — this is exactly what the Oliver Wyman framework envisions. Gifford has built a functioning hospital-to-hospital transfer protocol with UVMHC and Dartmouth Health that is already moving patients more efficiently through the system. Mt. Ascutney and Valley Regional Hospital have shared their CEO, CFO, and COO since late 2024, generating approximately \$600,000 annually in administrative savings. SVMC and RRMC are exploring shared specialty and support service lines. These are the early formations of the regional hub structure that Acts 167 and 68 envision, but they are bottom-up and incomplete rather than top-down and comprehensive.

The workforce crisis is present in every plan, but its texture varies by geography. SVMC's plans to reduce locum provider costs, Mt. Ascutney's Nursing Pathways Program funding two cohorts of nursing students at \$190,000, and Grace Cottage's near-elimination of travel nurses through local retention strategies all reflect the same underlying constraint: rural Vermont cannot recruit and retain clinical staff at the volume and cost structure that fee-for-service hospital operations require. The transformation plans' workforce sections are not primarily about hiring more people — they are about redesigning care models to require fewer high-cost clinicians per unit of patient outcome.

The analytics gap is an operational reality, not a theoretical concern. The AHS Hospital Transformation Analytical Support RFP, issued in late 2025 and awarded to McKinsey, was a state acknowledgment that Vermont does not yet have the analytics infrastructure to manage global budgets, measure population health outcomes at the hospital level, or track patient migration across its 14-institution system. The scope of that procurement — VHCURES integration, facility risk profiling, patient migration modeling, scenario modeling — reveals both the ambition of Vermont's transformation and the distance between current analytics capability and what AHEAD's launch in January 2027 will require.

The Transformation Plans: Vermont's System in Motion

The 148 individual transformation initiatives documented across Vermont's hospital plans in January 2026 are not policy proposals. They are operational commitments with named responsible parties, defined timelines, cost estimates, and measurement frameworks — produced under statutory obligation and reviewed by AHS staff who are rating their quality and tracking their progress. This is the Operations pillar working in real time.

Several initiatives merit specific attention because they illustrate the six-pillar interdependencies described in Chapter 1 with the granularity that only primary source documents can provide.

Grace Cottage Hospital's denial reduction program reduced insurance denials from \$76,160 in FY2024 to \$1,274 in FY2025 — a 98% reduction achieved through concurrent review for acute admissions, documentation improvement, and provider education. This is a direct illustration of the Operations-Economics dependency: better operational execution of billing and documentation produced \$75,000 in recovered revenue from care the hospital had already delivered but been unable

to collect. It also illustrates the workforce dimension — this improvement was driven by sustained teamwork between UR, coding, and billing staff, not capital investment.

Grace Cottage’s financial trajectory — from a \$4 million pre-audit operating loss in FY24 to approximately \$1 million in FY25 to projected break-even in FY26 — and its distinction as the only Vermont hospital that has received 4-star (now 5-star) ratings in the National Rural Health Star Rating System while receiving no DSH payments, makes it an analytically important counterexample to the narrative that Vermont’s smaller hospitals are uniformly failing. It is a demonstration that operational discipline and care model redesign can stabilize a small CAH without waiting for payment reform.

Mt. Ascutney’s shared senior leadership arrangement with Valley Regional Hospital — a shared CEO since December 2024, shared CFO since November 2025, and shared COO since January 2026 — is the book’s most concrete current example of the regionalization logic working at the administrative rather than clinical level. The \$600,000 annual savings at Mt. Ascutney alone partially offset the hospital’s obligation to subsidize the Historic Homes of Runnemedede residential care facility (\$950,000 in FY25) and a community pharmacy in Windsor that became the area’s only pharmacy after the town’s private pharmacy closed in 2025. The pharmacy subsidy is a direct illustration of the equity problem that rural hospital transformation must address: a small CAH bearing community infrastructure costs that its operating revenue cannot sustainably support.

Mt. Ascutney’s Community Health Team data — 1,132 individuals served over ten months in 2025, 80% of them over age 64, 22% requiring six or more contacts in the period — is the most concrete available illustration of Vermont’s demographic aging problem at the clinical level. The CHT is not serving a theoretical future population; it is already serving a population whose overwhelming predominance is geriatric, complex, and high-utilizing. The Age-Friendly Program that Mt. Ascutney is building with its transformation grant is an operational response to a present reality that the statewide demographic projections only describe in the aggregate.

The following table summarizes the transformation initiative landscape across all hospitals by theme, illustrating the relative distribution of transformation activity and the types of interventions Vermont’s system is deploying.

Initiative theme	Initiative count	Representative examples from hospital plans
Clinical service redesign	~55	SVMC RN Bridge to PCP; BMH MIH formalization; MAHHC Age-Friendly Program; Grace Cottage same-day access clinic; RPMC hospice utilization improvement
Regionalization / transfers	~20	NCH-NVRH shared regionalization planning; Gifford hospital-to-hospital transfers with UVMC/DHMC; MAHHC shared senior leadership with Valley Regional; SVMC-RPMC regional service lines (allergy, ENT, pain, ERCP)
Workforce development	~25	MAHHC Nursing Pathways Program (\$190K for two cohorts); SVMC 'grow our own' RN pipeline; Grace Cottage near-elimination of travel nurses; Mt. Ascutney SANE forensic capacity building
Technology and data	~20	SVMC Epic migration (\$30M); CVMC/Porter acuity management technology; Grace Cottage EMR interoperability interfaces; Grace Cottage ambient AI scribe (Coverys grant)

Shared purchasing / operational efficiency	~15	NECHN GPO launch (Gifford projected \$793K savings); captive insurance pool across NE Collaborative members; MAHHC-Valley Regional shared C-suite (\$600K annual savings)
Value-based care / payment alignment	~13	SVMC Belong Health Medicare Shared Savings ACO (2026); SVMC Medicaid global budget participation; Grace Cottage ACO participation and VATICA integration; Gifford FQHC model expansion

Figure 2.3. Vermont hospital transformation initiative themes, January 2026. Based on 148 initiatives from AHS Transformation Plans Analysis. Source: AHS Transformation_Plans_Analysis.xlsx (January 2026).

What the Plans Reveal About the System's Constraints

The transformation plans also document Vermont's system's constraints with a candor that formal policy documents rarely achieve. Three constraints are particularly important for subsequent chapters.

Interoperability remains a solved problem only on paper. Grace Cottage's plan documents the reality with precision: working with multiple regional partners who operate on different electronic medical records, "the process from order to completion is arduous and full of administrative waste" — relying on "antiquated and insecure technologies" because secure interoperability infrastructure does not yet connect Vermont's system. Vermont's HIE, VTTL, exists but is not functionally integrated with the workflows of most community hospitals. This is not a technology problem awaiting a solution — the solutions exist. It is an incentive and governance problem: no single hospital can justify the cost of building interoperability infrastructure, but the collective benefit to the system is substantial. This is a textbook Technology-Operations dependency failure.

Community pharmacy access has emerged as a new access gap. Mt. Ascutney is subsidizing Windsor's only pharmacy after the town's private pharmacy closed in 2025. Grace Cottage's pharmacy fills over 6,000 scripts monthly as a community service, collecting patients from Brattleboro who have lost local pharmacy access. These are not healthcare transformation issues in the traditional sense — but they illustrate the ecosystem of access barriers that Vermont's rural hospitals are increasingly absorbing. When a hospital becomes a community's pharmacy, its cost structure reflects that community function in ways that standard efficiency benchmarks do not capture.

The evidence that voluntary participation cannot achieve structural change runs through the plans like a thread. BMH's MIH program — one of the most clinically sophisticated primary care redesign initiatives in the state — explicitly identifies sustainable funding as its primary barrier, noting that standard billing cannot cover the cost and requesting that AHS provide reimbursement through RHTP funding. NVRH and NCH's regionalization work explicitly identifies legal counsel and project management as shared costs they cannot individually afford. Grace Cottage's interoperability improvement plan notes that "a consultant is needed" before costs can even be estimated. These are not capability failures — they are financing failures. The hospitals know what needs to be done. The current financial architecture makes it impossible to fund.

This is precisely the condition that Acts 167 and 68 are designed to address. The transformation plans submitted in January 2026 are the first detailed inventory of what Vermont's hospitals are attempting and what they cannot do without structural financial change. They are, in that sense, the

most important primary source Vermont’s transformation has yet produced — and the most direct evidence for the book’s central argument that single-pillar interventions are insufficient.

— *End of Chapter 2 draft additions* —

Sources: AHS Hospital_Tracking.xlsx; AHS Transformation_Plans_Analysis.xlsx; individual hospital transformation plans (Jan 15-31, 2026); GMCB Annual Report 2024; GMCB FY25 and FY26 Budget Orders; Oliver Wyman Act 167 Report (September 2024); WCAX/Vermont Business Magazine GMCB FY25 budget coverage; Mt. Ascutney Transformational Grant January 15 Report.